Announcement
Kindly note: We're currently experiencing a high volume of inquiries and calls, potentially leading to delays in our initial response.

Dermatology 23S Module 2

Go to Week 1 Academic Forum

Follow on Questions for week one

Settings

◄ Welcome to Module 2 Scenario 1 ►

Display replies in nested form

Follow on Questions for week one
by Derek Kayanja (Tutor) - Tuesday, 7 November 2023, 7:39 PM

Hi Group,

This is our last task for week one.

Please can you each answer **at least three** of the following questions, and **as a group** can we make sure that **all the questions below have been answered.**

1. Describe the five variants of angiokeratomas and their link with Fabry disease.
2. Discuss the clinical features of dermatofibrosarcoma protuberans and whether Mohs micrographic surgery or wide local excision should be used to treat it.
3. Discuss the clinical features of plexiform neurofibromas and the risk associated with them.
4. Should all dermatofibromas be biopsied or excised now there has been evidence showing that cellular dermatofibromas have malignant potential?
5. Discuss the clinical features and treatment of pyogenic granulomas.
6. Discuss the clinical features and treatment of digital myxoid pseudocyst.
7. Sebaceous naevus is associated with Schimmelpenning syndrome and phakomatosis pigmentokeratotica; discuss these conditions.
8. What are the other benign tumours associated with sebaceous naevus?
9. Discuss the clinical features of connective tissue naevi and the associated conditions.
10. Discuss aplasia cutis.

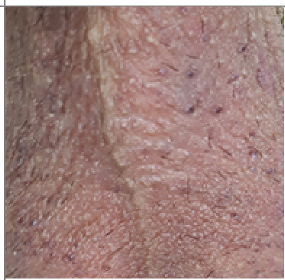
11. Discuss en coup de sabre.
12. Discuss the method used to excise an epidermoid cyst.
13. How does an angiolipoma differ from a lipoma?
14. What are the clinical features of Birt-Hogg-Dubé syndrome?
15. What are the clinical features of Bannayan-Riley-Ruvalcaba syndrome?
16. Discuss the clinical features and treatment of pilomatricomas.

223 words

Reply

Re: Follow on Questions for week one
by Pooran Outar - Wednesday, 8 November 2023, 7:06 AM

Question 1 - The 5 variants of angiokeratomas and their link with Fabry disease.

Type	Epidemiology	Clinical features	Related Image
Solitary or Multiple angiokeratoma	Most common type Male predominance	Small, warty, black papules 2–10 mm in diameter Mostly on lower limbs, but may be seen anywhere on the body. Trauma or chronic irritation favours new lesions.	 (Coulson, 2021)
Angiokeratoma of Fordyce	2nd most common type, Male predominance Increases with age	Multiple well-circumscribed dome-shaped papules 2–5 mm in diameter Mainly on scrotum and vulva.	 (The Dermatology Clinic London , 2022)
Angiokeratoma of Mibelli	Rare Autosomal dominant inheritance pattern Appears in childhood or adolescence	Multiple warty purple papules; mostly on dorsal and lateral sides of the fingers and toes hands feet, elbows, knees and breast.	 (Coulson, 2021)

Angiokeratoma circumscriptum	Rare Congenital or acquired Female predominance	Band like appearance Affects mostly lower limbs but can appear anywhere including the tongue.	 (Coulson, 2021)
Angiokeratoma corporis diffusum **(Fabry disease)**	Rare X-linked genetic disorder Male predominance Less severe in carrier females	Cluster of small, dark red spots on the skin. Heat intolerance and hypohidrosis Acroparesthesia Cloudiness of the cornea Hearing loss Abdominal symptoms Recurrent fever, fatigue Most numerous over buttocks, groin, and lower trunk (Bikini Pattern).	 (Demczko , 2022)

"In Fabry disease, angiokeratomas are caused by the accumulation of Gb3 in the dermal endothelial cells, which leads to bulge and incompetence of the vessel walls" (Nutan, 2019).

References:

- Coulson, I. (2021). *Angiokeratoma: Types and Appearances — DermNet*. [online] dermnetnz.org. Available at: https://dermnetnz.org/topics/angiokeratoma.
- Demczko , M. (2022). *Fabry Disease - Pediatrics*. [online] MSD Manual Professional Edition. Available at: https://www.msdmanuals.com/professional/pediatrics/inherited-disorders-of-metabolism/fabry-disease [Accessed 8 Nov. 2023].
- Jin, Q. (2019). Fabry disease | DermNet. [online] dermnetnz.org. Available at: https://dermnetnz.org/topics/fabry-disease#:~:text=In%20Fabry%20disease%2C%20angiokeratomas%20are.
- Nutan, F. (2019). Angiokeratoma Corporis Diffusum (Fabry Disease) Clinical Presentation: History, Physical Examination, Complications. [online] emedicine.medscape.com. Available at: https://emedicine.medscape.com/article/1102964-clinical [Accessed 8 Nov. 2023].
- Sharquie , K. and Jabbar, R.I. (2020). 2021.2.5.angiokeratoma. [online] Our Dermatology Online. Available at: http://www.odermatol.com/issue-in-html/2021-2-5-angiokeratoma/ [Accessed 8 Nov. 2023].
- The Dermatology Clinic London (2022). *Angiokeratoma of Fordyce Treatment UK | The Dermatology Clinic London*. [online] The Dermatology Clinic. Available at: https://thedermatologyclinic.london/skin-conditions/angiokeratoma-treatment-london/ [Accessed 8 Nov. 2023].

350 words

Reply

Re: Follow on Questions for week one
by Derek Kayanja (Tutor) - Saturday, 11 November 2023, 3:31 PM

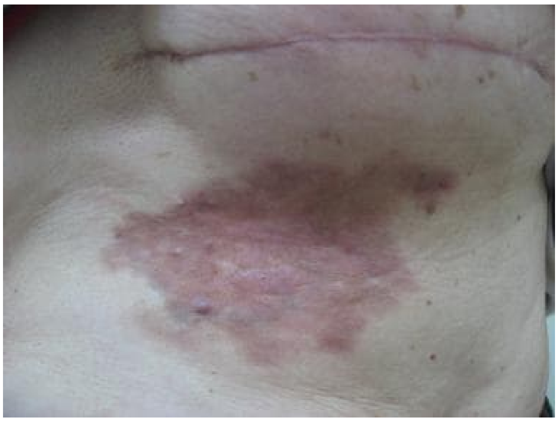
Pooran, thank you very good post.

6 words

Reply

Re: Follow on Questions for week one
by Pooran Outar - Wednesday, 8 November 2023, 7:23 AM

Question 2 - The clinical features of dermatofibrosarcoma protuberans and Surgical Care



According to Maddan, 2020 Dermatofibrosarcoma protuberans usually present as:

- Large, indurated plaque several centimetres in diameter.
- Firm, irregular nodules varying in colour from flesh to reddish brown.
- Mostly mobile but fixation to deeper structures may occur in the later stage of the tumour.
- Telangiectasia may be apparent on the surface or at the periphery.
- In some cases, DFSP may appear as a morphealike, atrophic, and sclerotic plaque that is purplish in color, lacking nodules, and may eventually develop ulcers as it gradually increases in size.
- Most commonly appears on the trunk and proximal extremities

Considerations for Surgical Care
The choice between these two techniques depends on factors such as the tumour's size, location, and the availability of Mohs surgery in a given area. Mohs surgery has a superior cure rate and is preferred for head and neck cases. Some Mohs surgeons may take an additional layer of tissue for further examination or immunostaining to enhance the cure rate. Alternatively, modified Mohs techniques may be used in some cases.

In Wide Local Excision approach, the tumour and a wide margin of healthy tissue (2-3 cm or more beyond the clinically identifiable tumour border, including the fascia) are removed. Despite wide local excision, DFSP can have a relatively high recurrence rate.

Mohs surgery is gaining acceptance as the preferred treatment method. It involves removing the tumour in layers while examining the margins microscopically. This method requires less tissue removal and allows for complete margin assessment. It is especially recommended for lesions on the head and neck region, where tissue conservation is crucial. However, Mohs surgery can be time-consuming, especially for large tumours (Maddan, 2020).

Reference:
Madan, R.K. (2020). Dermatofibrosarcoma Protuberans Clinical Presentation: History, Physical Examination. [online] emedicine.medscape.com. Available at: https://emedicine.medscape.com/article/1100203-clinical#b2 [Accessed 8 Nov. 2023].

304 words

Reply

Re: Follow on Questions for week one
by Derek Kayanja (Tutor) - Saturday, 11 November 2023, 3:32 PM

Thank you Pooran, good post.

5 words

Reply

Re: Follow on Questions for week one
by Pooran Outar - Wednesday, 8 November 2023, 7:55 AM

Question 5 - Clinical features and treatment of pyogenic granulomas.

Clinical features of pyogenic granuloma (Sivasaththivel, Tandon and Ly, 2021):

- Painless red fleshy nodule
- Typically 5-10mm in diameter
- Rapid growth over a few weeks.
- The surface is initially smooth but can ulcerate, become crusty, or verrucous.
- Usually solitary, but multiple nodules and satellite lesions can erupt.
- Commonly appears on the fingers, face and oral mucousa.
- Bleeds easily with minor trauma.

Treatment:

According to the British Association of Dermatologists, 2008, treatment options include:

- Topical treatment (Imiquimod cream 5%, Timolol gel 0.5%, Silver Nitrate, Intralesional steroid injection, Cryotherapy)
- Procedural treatment (Laser, Curettage and cautery, Surgical excision, Vascular and ablative)

References:

- Sivasaththivel, M., Tandon, S. and Ly, L. (2021). Pyogenic granuloma | DermNet NZ. [online] Dermnetnz.org. Available at: https://dermnetnz.org/topics/pyogenic-granuloma.lasers
- The British Association of Dermatologists (2008). *British Association of Dermatologists*. [online] www.bad.org.uk. Available at: https://www.bad.org.uk/pils/pyogenic-granulomas/.

142 words

Reply

Re: Follow on Questions for week one
by Derek Kayanja (Tutor) - Saturday, 11 November 2023, 3:32 PM

Thank you Pooran, good post and nice images.

8 words

Reply

Re: Follow on Questions for week one
by Pooran Outar - Wednesday, 8 November 2023, 8:06 AM

Question 6 - The clinical features and treatment of digital myxoid pseudocyst.

Clinical features of Digital Myxoid Cyst:

- Shiny smooth papule found at the end of a finger or toe, close to the nail.
- Not surrounded by a capsule, unlike a true cyst
- A Jelly-like sticky fluid sometimes tinged with blood may be expressed

Treatment:

Despite the following treatment options by Oakley, 2016, digital myxoid pseudocyst often reoccurs:

- Repeatedly pressing firmly on the cyst
- Squeezing out its contents (make a hole with a sterile needle)
- Cryotherapy (freezing)
- Steroid injection
- Sclerosant injection
- Surgical removal.

Reference:

Oakley, A. (2016). Digital mucous (myxoid) cyst | DermNet NZ. [online] dermnetnz.org. Available at: https://dermnetnz.org/topics/digital-myxoid-pseudocyst.

109 words

Reply

Re: Follow on Questions for week one
by Derek Kayanja (Tutor) - Saturday, 11 November 2023, 3:33 PM

Thank you Pooran, good post.

5 words

Reply

Re: Follow on Questions for week one
by Bhavika Patel - Sunday, 12 November 2023, 10:28 PM

Hi Pooran,

Thank you for your detailed answers to the some of the questions, I wanted to add to your post regarding digital myxoid pseudocysts.

Digital myxoid pseudocyst are solitary soft fluid-filled papules or nodules “overlying the distal interphalangeal joints of a finger” or toe (Bardach, 1983). Causes include osteoarthritis and abnormal amounts of mucin deposits in the skin resulting in cyst formation (Oakley, 2016).

I have personal experience of dealing with these lesions within my minor surgery clinics. In practice, I have found that a needle aspiration of the inclusion cysts next to nailbeds have better cosmetic results however higher risk of recurrence. Cryotherapy was not found to be useful due to poor patient tolerance for longer treatment times, and risk of nailbed blistering. I have not tried excising so close to the nailbed but would rather avoid that option where possible. I found in my research that in patients with osteoarthritis, a corticosteroid injection into the affected joints without affecting the tendons, is a more acceptable approach than surgical intervention however there continues to be that risk of recurrence (Patel, 2022).

Bardach, H. G. (1983) Managing Digital Mucoid Cysts by Cryosurgery with Liquid Nitrogen: Preliminary Report. The Journal of Dermatologic Surgery and Oncology. [Online] 9 (6), 455–458. (Accessed 12th November 2023)

Oakley, A. (2016). Digital myxoid pseudocyst. DermNet. [Online] Available at https://dermnetnz.org/topics/digital-myxoid-pseudocyst (Accessed 12th November 2023)

Patel, R. A., Ariza-Hutchinson, A., Emil, N. S. et al. (2022) Intraarticular injection of the interphalangeal joint for therapy of digital mucoid cysts. Rheumatology international. [Online] 42 (5), 861–868. (Accessed 12th November 2023)

263 words

Reply

Re: Follow on Questions for week one
by Xiao Liey Ding - Thursday, 9 November 2023, 2:55 PM

Question 12

Several surgical approaches exist for treating epidermoid cysts. Although complete surgical excision is effective in ensuring sac removal and preventing recurrence, it is a time-consuming procedure that necessitates suture closure.

An alternative, less invasive method known as the minimal excision technique has been suggested and proven successful. This approach involves making a 2- to 3-mm incision, expressing the cyst contents, and extracting the cyst wall through the incision. To aid in sac removal, vigorous finger compression is applied to express the cyst contents and separate the cyst wall from surrounding tissues. Closure of the small wound can be achieved with a single suture, although many physicians opt not to close the incision. A modified version of this technique involves using a punch biopsy instrument to create the opening into the cyst.

Reference:

Thomas J Zuber (2002). Minimal Excision Technique for Epidermoid (Sebaceous) Cysts. *Am Fam Physician*. 65(7):1409-1412

Question 13

Angiolipoma is a variant of lipoma with additional vascular component and is benign.

	Lipoma	Angiolipoma
Composition	Mature fat cells	Fat cells with blood vessels
Appearance	Soft, lobulated mass	May have slightly different feel due to presence of blood vessels
Symptoms	Typically, asymptomatic	Can sometimes be tender or painful.

Location	Located superficially in subcutaneous tissue. But can also appear deep to the fascia	Occurs in subcutaneous or intra-muscular.

Reference:

Marta Sbaraglia and Angelo Paolo Dei Tos (2019). Adipocytic Tumours. *Practical Soft Tissues Pathology: A Diagnostic Approach.* Page 311-340
https://doi.org/10.1016/B978-0-323-49714-5.00012-0

239 words

Reply

Re: Follow on Questions for week one
by Adam Tollitt - Thursday, 9 November 2023, 7:51 PM

Hi Ding,

Thank you for your comprehensive answer to question 12.

To expand further on your answer, I found one randomised study that concluded improved surgical outcomes (measured by the mean length of surgical wound and procedure time) with the minimal incision technique compared to complete surgical excision for small epidermoid cysts located on the head (Alijanpour, Alijanpour and Alijanpour, 2018). Interestingly in this study there was no statistically significant difference in rates of cyst recurrence between the two surgical approaches. Another smaller study also suggests that the minimal incision technique can be used to successfully treat smaller facial epidermal cysts (0.5-1cm) in size but was less successful for larger cysts (>1.1cm) (Hj and Kc, 2009).

From my literature search I found limited evidence that conclusively recommends either approach as being superior in terms of surgical outcome or rates of cyst recurrence.

I also came across an interesting study that proposed a novel approach to the minimal incision technique using a CO2 laser to create a hole in the cyst wall to allow cyst extraction (Zhang et al., 2020).

References

Alijanpour, A., Alijanpour, K. and Alijanpour, K. (2018). Comparison of the Surgical Outcomes of Minimal Excision and Elliptical Excision Techniques in Treating Epidermal Inclusion Cysts: A Prospective Randomized Study. Shiraz E-Medical Journal, In Press(In Press). doi:https://doi.org/10.5812/semj.55936

Hj, Y. and Kc, Y. (2009). A new method for facial epidermoid cyst removal with minimal incision. Journal of the European Academy of Dermatology and Venereology, 23(8), pp.887–890. doi:https://doi.org/10.1111/j.1468-3083.2009.03191.x.

Zhang, L., Hao, L., Tan, J.-X., Huang, Y., Huang, H., Hu, J. and Bi, M. (2020). Efficacy of the combination of minimally invasive CO2 laser incision with photodynamic therapy for infected epidermoid cysts. Photodiagnosis and Photodynamic Therapy. doi:https://doi.org/10.1016/j.pdpdt.2020.101791.

284 words

Reply

Re: Follow on Questions for week one
by Derek Kayanja (Tutor) - Saturday, 11 November 2023, 3:43 PM

Thank you Adam for this interesting post. Minimal excision has the advantage of of giving better cosmetic results than standard excision.

21 words

Reply

Re: Follow on Questions for week one
by Derek Kayanja (Tutor) - Saturday, 11 November 2023, 3:34 PM

Thank you Xiao Liey, good post.

6 words

Reply

Re: Follow on Questions for week one
by Rahima Abdulaziz Ahmed - Sunday, 12 November 2023, 2:24 PM

Thank you Ding for the table , just to add in alittlebit;
Like u mentioned Angiolipomas are a variant of lipomas but the only difference is that ,Angiolipomas contains not only fat cells but cells but also blood vessels , giving it a more vascularized appearance compared to a typical lipoma. While both are generally non cancerous , angiolipomas may be more painful due to the presence of blood vessels and are often firmer to touch. Dr.Cheng (2013)
Sonographically , Angiolipomas show heterogenous hyperechoic ovoid masses with lesser visualized lateral tumor capsules whereas superficial lipomas appear as homogenous isoechoic spindle shaped masses with well visualized lateral capsules.

References:
Shin . J, Kim. J and Park.S (2016) PubMed *Sonographic Differentiation Between Angiolipomas and Superficial Lipomas* (online) Available at https://pubmed.ncbi.nlm.nih.gov/27738296/ (Accessed on 12/11/23)
Dr.Cheng.H (2013) DermNet *Angiolipoma* (online) Available at https://dermnetnz.org/topics/angiolipoma (Accessed on 12/11/23)

143 words

Reply

Re: Follow on Questions for week one
by Derek Kayanja (Tutor) - Sunday, 12 November 2023, 2:32 PM

Rahima, thank you for your post (please remember that one shouldnt use titles in the in text citations).

18 words

Reply

Re: Follow on Questions for week one
by Felica Mwinji Namukonde - Thursday, 9 November 2023, 3:00 PM

Connective Tissue Nevi

This lesion occurs when deeper layers of the skin do not develop correctly or there components of these layers occur wrong this is also a very uncommon condition these hamartomas arise in reticular dermis they include;
elastomas (most common)
collagenomas
mixed lesions

Clinical Features;
they are usually painless, scattered, flesh-colored or yellowish papules or nodules 2-10mm in size
they maybe congenital or acquired that progress with time to adolescence. mostly distributed symmetrically mostly on trunk and volar surfaces of proximal extremities.
Localized plaques are usually on the trunk and limbs in segmental or linear pattern

Associated conditions include;
Buschke-ollendorf Syndrome; rare hereditary disorder which occurs with increased accumulation of elastin in dermis sometimes lesions maybe present at birth but usually appear after first year of life, firm yellowish wrinled nodules which group together in plaques, mostly appear on abdomen, buttocks arms or thighs.

Elastosis Perforans Serpignosa;
a perforating disorder abnormal elastin fibres are extruded through epidermis, appears in adolescence affects more than one site of face, neck or arms.

References;
McCuaig CC, Miedzybrodzki B (2023) *Uptodate* Available at; *https://www.uptodate.com/contents/buschke-ollendorff-syndrome?search=connective%20tissue%20nevus&source=search_result&selectedTitle=1~11&usage_type=default&displa*
Accessed; (9/11/2023)
Ngan V (2003) Available at: *https://dermnetnz.org/topics/connective-tissue-naevi* Accessed (9/11/2023)

195 words

Reply

Re: Follow on Questions for week one
by Derek Kayanja (Tutor) - Saturday, 11 November 2023, 3:43 PM

Thank you Felica, good post.

5 words

Reply

Re: Follow on Questions for week one
by Ayten Sadek - Thursday, 9 November 2023, 8:24 PM

Question 14:

Birt-Hogg-Dubé syndrome (BHDS) is a rare genetic autosomal dominant disorder characterized by the development of benign skin tumors called fibrofolliculomas(tumor in hair follicle), Trichodiscoma(tumor of hair disc), Angiofibroma(vascular tumor) and Acrochordon(skin tags)

In addition to developing lung cysts, and an increased risk of certain types of kidney tumors,specifically renal cell carcinoma and spontaneous pneumothorax due to the pulmonary cysts.

1. Fibrofolliculomas: These are the hallmark skin lesions of BHDS. Fibrofolliculomas are small, firm, dome-shaped bumps that commonly appear on the face, neck, and upper trunk.

Other Skin Lesions: In addition to fibrofolliculomas, individuals with BHDS might develop other skin lesions, such as trichodiscomas (small, smooth, flesh-colored papules) and acrochordons (skin tags).

Fibrofolliculomas in Birt Hogg Dube syndrome

Image available at: https://dermnetnz.org/topics/fibrofolliculomas-in-birthoggdube-images

2-Pulmonary Cysts: patients often develop cysts in the lungs and they are at an increased risk of experiencing spontaneous pneumothorax, especially in early adulthood .

3. Renal Tumors:It is associated with an elevated risk of developing various types of kidney tumors, most commonly chromophobe renal cell carcinoma and oncocytoma.

4. Spontaneous pneumothorax: The presence of lung cysts can increase the risk of spontaneous pneumothorax.

References:

dermnetnz.org(2005) Birt-Hogg-Dubé syndrome (online) available at: https://dermnetnz.org/topics/birt-hogg-dube-syndrome(accessed on 9/11/2023)

emedicine.medscape(2021)) Birt-Hogg-Dubé syndrome (online) available at: https://emedicine.medscape.com/article/1060579-clinical#b2(accessed on 9/11/2023)

208 words

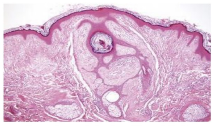
Reply

Re: Follow on Questions for week one
by Oyeombiliyetu Jason - Saturday, 11 November 2023, 12:54 PM

Clinical features of Birt- Hogg- Dube syndrome in addition I would like to add.

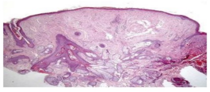
Cutaneous manifestation

1. Fibrofolliciloma



(V. Lopez 2011)

2. Trichodiscoma



(V. Lopez 2012)

3. Acrochordons

Menko et al. Proposed Diagnostic criteria

(patient should have 1major or 2minor criteria)

“Major criteria: 1. At least 5 fibrofolliculomas or trichodiscoma, at least 1 histologically conformed and of adult onset

2. FLCN germline mutation

Minor criteria: 1. Multiple bilateral basically located lung cysts (with no known cause)

2. Renal cancer- early onset (<50yo) or multifocal or bilateral or renal cancer of mixed cheomophole, and oncocystic histology.

3. A firs-degree relative with BHDS.”

Systemic manifestations: pulmonary cysts, thyroid cancer, parotid oncocytoma, basal lung cyst and parathyroid adenomas.

References

1.Jonathan S. Crane et al (2023) ‘Birt-Hogg-Dube Syndrome’ *National Library of Medicine.*available at: https://www.ncbi.nlm.nih.gov/books/NBK448061/#:~:text=Birt%20Hogg%20Dube%20syndrome%20is,lu Accessed: 11 Novermber 2023.

2. V. Ngan et al. (2005) ‘Birt-Hogg-Dube Syndrome’ *Dermnet.* Available at: https://dermnetnz.org/topics/birt-hogg-dube-syndrome. Accessed: 11 November 2023.

3. N. Gupta et al. (2013). ‘Pulmonary manifestations of Birt-Hogg-Dube Syndrome’. *National Library of Medicine.*12(3):387-396. Available at: https://pubmed.ncbi.nlm.nih.gov/23715758/. Accessed: 11 November 2023.

4. V. Lopez et al. (2012) ‘Birt-Hogg-Dube Syndrome: An Update’ *Novelties in Dermatology.* 103(3): 198-206.available at: https://www.actasdermo.org/en-birthoggdube-syndrome-an-update-articulo-S1578219012001011. Available: 11 November 2023.

193 words

Reply

Re: Follow on Questions for week one
by Derek Kayanja (Tutor) - Saturday, 11 November 2023, 3:46 PM

Oyeombiliyetu, thank you for your addition to our discussion.

9 words

Reply

Re: Follow on Questions for week one
by Derek Kayanja (Tutor) - Saturday, 11 November 2023, 3:44 PM

Thank you Ayten, good post.

5 words

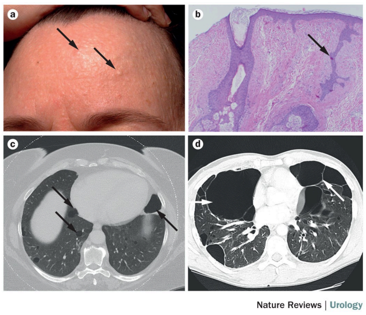
Reply

Re: Follow on Questions for week one
by Sadaf Basharat - Thursday, 9 November 2023, 9:00 PM

Q14: What are the clinical features of Birt-Hogg-Dubé syndrome?

Birt–Hogg–Dubé syndrome is a rare genetic condition which manifests as benign tumors involving the head, face, and upper body, including fibrofolliculoma, trichodiscoma, angiofibroma, and acrochordon. BHD is associated with an increased risk of colon or kidney cancer (15%) and spontaneous pneumothorax from pulmonary cysts. Also termed fibrofolliculoma with trichodiscoma and acrochordon, it was first described by Canadian physicians Birt, Hogg, and Dubé in 1977.

BHG Syndrome usually comprises of smooth, painless, small papular skin lesions which gradually appear on the scalp, face, neck, chest, and back, typically seen around ages 30 or 40. These lesions, identified as fibrofolliculomas (trichodiscomas) and acrochordons (skin tags), are dome-shaped bumps or soft 1–2 mm warts with thin necks. Their numbers can vary from a couple to numerous, and once formed, they are permanent. Additionally, oral mucosal polyps, collagenomas, angiolipomas, and deforming lipomas may also manifest.
Ngan,V.and Writer, N. (2005)



Refernces:

Ngan,V.and Writer, N. (2005),*Birt-Hogg-Dubé Syndrome,* DermNet NZ [online] Available at https://dermnetnz.org/topics/birt-hogg-dube-syndrome.

Schmidt, L.S. and Linehan, W.M., 2015. Clinical features, genetics and potential therapeutic approaches for Birt–Hogg–Dubé syndrome. *Expert opinion on orphan drugs*, *3*(1), pp.15-29.

194 words

Reply

Re: Follow on Questions for week one
by Sadaf Basharat - Thursday, 9 November 2023, 9:29 PM

15. What are the clinical features of Bannayan-Riley-Ruvalcaba syndrome?

Bannayan–Riley–Ruvalcaba syndrome (BRRS) is an autosomal dominant condition characterised by macrocephaly, developmental delay and subtle cutaneous features including multiple non cancerous tumors, facial papilomas, lipomas and hamartomas along with genital lentigines . Developmental delays are also associated with BRRS.
Other Clinical features include acanthosis nigricans and multiple acrochordons.

References:

Lynch, N.E., Lynch, S., McMenamin, J. and Webb, D., 2009. Bannayan–Riley–Ruvalcaba syndrome: a cause of extreme macrocephaly and neurodevelopmental delay. *Archives of disease in childhood*, *94*(7), pp.553-554.
Hendriks, Y.M.C., Verhallen, J.T.C.M., Van der Smagt, J.J., Kant, S.G., Hilhorst, Y., Hoefsloot, L., Hansson, K.M., Van der Straaten, P.J.C., Boutkan, H., Breuning, M.H. and Vasen, H.F.A., 2003. Bannayan–Riley–Ruvalcaba syndrome: further delineation of the phenotype and management of PTEN mutation-positive cases. *Familial cancer*, *2*, pp.79-85.

16. Discuss the clinical features and treatment of pilomatricomas:

Pilomatricomas is a rare harmless hair follicles tumor derived from hair matrix cells. It is also known as pilomatrixzoma.
Its usually seen in children but can affect adults as well and affects females more than males.
It is associated with myotonica dystrophica.
Clinical features include a solitary skin-colored or purplish lesion.
- Commonly found on the head and neck but can occur on any part of the body.
- While often asymptomatic, it can present as been tender.
- They may remain unchanged for years or gradually increase in size, reaching several centimeters.
- Pilomatricoma is characterized by calcification, resulting in a hard, bony feel and often forming an angulated shape known as the 'tent' sign.

Treatment usually requires surgical excision but can re occur if not excised completely. They usually do not go without surgery.
References:

Oakley A.,(2017). Pilomatricoma| DermNet NZ [online] Available at: https://dermnetnz.org/topics/pilomatricoma
Zaman, S., Majeed, S. and Rehman, F., 2009. PILOMATRICOMA œ STUDY ON 27 CASES AND REVIEW OF LITERATURE. *Biomedica*, *25*.

311 words

Reply

Re: Follow on Questions for week one
by Derek Kayanja (Tutor) - Saturday, 11 November 2023, 3:46 PM

Sadat, thank you for your good posts.

7 words

Reply

Re: Follow on Questions for week one
by Ayten Sadek - Thursday, 9 November 2023, 10:35 PM

Question 15:

Bannayan-Riley-Ruvalcaba syndrome (BRRS) is a rare genetic disorder characterized by multiple hamartomas and certain developmental and intellectual disabilities. Here are the clinical features associated with Bannayan-Riley-Ruvalcaba syndrome:

1. Macrocephaly: Individuals with BRRS often have abnormally large size head

2. Multiple Hamartomas: Hamartomatous polyposis in the gastrointestinal tract, Polyps in the gastrointestinal tract are common in BRRS.

3. Developmental Delay: Children may experience delays in reaching developmental milestones, such as walking and talking.

4. Muscle Hypotonia: It can cause decreased muscle tone (hypotonia)

5. Dermatologic Manifestations: Skin abnormalities, such as dark freckling on the penis and genital area (lentigines), may be present. : Genital Lentigines, which are dark freckles, can appear on the genital area,These skin changes are characteristic of BRRS. In addition to Facial papillomas, vascular malformations, lipomas, acanthosis nigricans. Some individuals with BRRS develop lipomas.

Reference:

Rarediseases.org(2021) Bannayan-Riley-Ruvalcaba syndrome(online) available at: https://rarediseases.org/rare-diseases/ruvalcaba-syndrome/#disease-overview-main (accessed on 9/11/2023)

Question 16:

Clinical Features of Pilomatricomas:

Pilomatricomas, also known as calcifying epitheliomas of Malherbe, are hair follicle tumors. They are relatively common in children and adolescents, but they can appear at any age. Here are the clinical features of pilomatricomas:

They are Firm, Painless Nodules, commonly on the face, neck, or upper arms but they may occur at any site. They can range in size from a few millimeters to several centimeters. They tend to grow slowly over time. Pilomatricomas have a distinctive hard or rock-like consistency due to the calcification within the tumor which often results in an angulated shape (the tent sign). It is usually the color of the normal skin, but reddish-purple overlying skin may occur and the overlying skin can be stretched and thinned

Image available at:

https://dermnetnz.org/topics/pilomatricoma

Treatment of Pilomatricomas:

The primary and most effective treatment for pilomatricomas is surgical excision (removal) of the tumor. This is usually done under local anesthesia. Complete removal is curative, and recurrence after successful excision is rare.

References:

dermnetnz.org(2017) Pilomatricoma(online) available at: https://dermnetnz.org/topics/pilomatricoma (accessed on 9/11/2023)

325 words

Reply

Re: Follow on Questions for week one
by Derek Kayanja (Tutor) - Saturday, 11 November 2023, 3:47 PM

Thank you Ayten, good posts.

5 words

Reply

Re: Follow on Questions for week one
by Xiao Liey Ding - Friday, 10 November 2023, 2:49 PM

Question 11

En coup de sabre is a descriptive term used to refer to a specific manifestation of localized scleroderma(Luther E. et al,2020), a rare autoimmune disorder that affects the skin and underlying tissues. The term itself is French for "sword strike," reflecting the characteristic appearance of the skin changes associated with this condition.

En coup de sabre primarily affects the face and forehead. The hallmark feature is a linear or band-like area of atrophy on the forehead or scalp, resembling the mark left by a sword. This area may extend vertically and can involve the underlying tissues, including muscle and bone. The affected skin may become tight, shiny, and atrophic, with loss of elasticity. Sclerosis refers to the hardening and thickening of the skin. Changes in pigmentation may occur in the affected area, leading to lighter or darker skin compared to the surrounding tissue. Hair loss (alopecia) can occur within the affected area.

Some individuals with en coup de sabre may experience neurological symptoms. These can include headaches and, less commonly, seizures. The condition can affect not only the skin but also the underlying neural structures and causing vision problems.

Reference:

Luther,E., Mitchell,G., Oakley,A (2020). *Morphoea en coup de sabre.* DermNet [online]. Available at: https://dermnetnz.org/topics/morphoea-en-coup-de-sabre. (Accessed: 10 Nov 2023)

210 words

Reply

Re: Follow on Questions for week one

by Derek Kayanja (Tutor) - Saturday, 11 November 2023, 3:47 PM

Xiao Liey, thank you good post.

6 words

Reply

Re: Follow on Questions for week one

by Rahima Abdulaziz Ahmed - Sunday, 12 November 2023, 5:12 PM

Q10;

APLASIA CUTIS:

Is a rare congenital disorder characterized by the absence of skin, typically on the scalp , but can occur on other parts of the body as well. (Adigun 2020)

Cause; The exact cause is unclear and it maybe multifactorial. Genetic factors , or disruptions during embryonic development have been proposed as possible contributors.

Presentation:

- appear at birth as a small, well-defined, non inflammatory area of missing skin, or it can more extensive , involving larger portions of the scalp or other body parts .The absence of skin can expose underlying structures such as bone or dura mater (the outermost membrane covering the brain)

-Superficial aplasia cutis involves only the epidermis (upper layers of skin)and shallow defects usually heal over before the child is born, leaving a scar.

-Deeper defects can extend through the dermis, subcutaneous tissue, and rarely periosteum, skull, or dura.

-Aplasia cutis may partially heal before delivery and appear as a hairless, atrophic, membranous and parchmentlike or fibrotic scar.

-Some people with aplasia cutis may also have other congenital malformations of the heart, gastrointestinal, genitourinary (such as gastroschisis or omphalocele), and central nervous systems (such as meningomyelocele or spinal dysraphism). (Ngan and Writer, 2003

Variability:

Aplasia cutis can vary widely in severity and may occur in isolation or as part of a syndrome with associated anomalies affecting other organ systems.

Treatment:

Management depends on the extent and severity of the condition . In milder cases , the lesion may heal spontaneously over time . More extensive cases may require surgical intervention to repair the affected area and protect underlying structures.

Prognosis:

It varies , with many cases having a good outcome , especially if the defect is small. However , larger or more complex cases maybe associated with complications or long term issues.

Complications:

Major complications are rare , but may include ;hemorrhage, secondary local infection, meningitis, or sagittal sinus thrombosis.(Adigun 2020)

Larger lesions associated with underlying bony defects may result in death secondary to central nervous system infection or hemorrhage from the sagittal sinus.

References;

Ngan. V and Writer. S (2003) DermNet *Aplasia cutis congenita* (online) Available at: https://dermnetnz.org/topics/aplasia-cutis. (Accessed on 12/11/23)

Adigun.G (Jun 16, 2020)Medscape *Aplasia Cutis Congenita* (online) Available at: https://emedicine.medscape.com/article/1110134 (Accessed on 12/11/23)

what other conditions/ syndromes is aplasia cutis associated with ?

what are the differential diagnosis of aplasia cutis

394 words

Reply

Re: Follow on Questions for week one

by Sophie Holloran - Sunday, 12 November 2023, 10:03 PM

Question 8: What are the other benign tumours associated with sebaceous naevus?

These are:

Viral warts

Trichoblastoma

Syringocystadenoma papilliferum

Tricholemmoma

Sebaceoma

Nodular hidradenoma

Hidrocystoma

Eccrine poroma

Reference:

1. Wright TS - Up To Date (2023) Nevus sebaceus and nevus sebaceus syndromes. Available at: https://www.uptodate.com/contents/nevus-sebaceus-and-nevus-sebaceus-syndromes?source=history_widget (Accessed 12/11/23)

46 words

Reply

Re: Follow on Questions for week one

by Sophie Holloran - Sunday, 12 November 2023, 10:16 PM

Question 7: Sebaceous naevus is associated with Schimmelpenning syndrome and phakomatosis pigmentokeratotica; discuss these conditions.

Schimmelpenning syndrome:

- The association of a linear nevus sebaceus which are often multiple and follow the lines of Blaschko
- Can be associated with cerebral, occular and skeletal deficits
- All reported cases are sporadic as the syndrome is caused by a fatal autosomal dominant mutation that survives by somatic mosaicism.

Schimmelpenning syndrome is also known as the following:

- Schimmelpenning-Feuerstein-Mims syndrome
- SFM syndrome

- Linear sebaceous naevus syndrome
- Jadassohn naevus phakomatosis
- Naevus sebaceous of Jadassohn
- Organoid naevus phakomatosis.

Phacomatosis pigmentokeratotica

- Presentation of linear sebaceous naevus with a papular naevus spilus (speckled lentiginous neavus) - this presents as a cafe au lait macule in a baby
- Caused by HRAS mutation
- Extra-cutaneous features include neurological, musculoskeleal, endocrine and vascular abnormalities
- Associated with an increased risk of cutaneous and internal malignancies

References:

1. Wright TS - Up to Date (2023) *Nevus sebaceus and nevus sebaceus syndromes.* Available at: https://www.uptodate.com/contents/nevus-sebaceus-and-nevus-sebaceus-syndromes?source=history_widget#H3942155852 (Accessed 12/11/23)

2. Oakley - DermNet NZ (2010) *Epidermal naevus syndromes.* Available at: https://dermnetnz.org/topics/epidermal-naevus-syndromes (Accessed 12/1//23)

174 words

Reply

Re: Follow on Questions for week one
by Zarmeena Khan - Wednesday, 15 November 2023, 5:22 PM

Aplasia cutis is a rare congenital disorder characterized by the absence of a portion of skin, usually on the scalp, but it can occur on other parts of the body as well. This condition is present at birth and can vary widely in its severity, ranging from a small, superficial defect to a more significant absence of skin, sometimes exposing the underlying bone or other structures.

Several factors can contribute to the development of aplasia cutis, but the exact cause is often unknown. Some potential factors include genetic predisposition, vascular problems, teratogenic drugs, and environmental factors. Aplasia cutis can occur as an isolated condition or be associated with other congenital anomalies or genetic syndromes.

The presentation of aplasia cutis can be diverse. The lesions are typically round or oval and may vary in size. They can appear as small, shallow ulcers or larger, more significant defects. The absence of skin can sometimes be covered by a thin membrane or a scab. The severity of the condition can determine the appropriate management.

Treatment of aplasia cutis depends on the size and location of the skin defect. Small lesions may heal on their own with basic wound care, while larger or more complicated cases may require surgical intervention. In some instances, a skin graft may be necessary to cover the affected area and promote healing.

The prognosis for individuals with aplasia cutis depends on the size and severity of the skin defect, as well as any associated abnormalities or complications. With appropriate medical care, many individuals with aplasia cutis can have a good outcome, but this can vary from case to case.

(Pimenta, Lapa and Ramos, 2017)

Because aplasia cutis is a rare condition, and its causes can be complex, it is important for affected individuals to be evaluated and managed by a multidisciplinary team of healthcare professionals, including geneticists, dermatologists, and surgeons, to provide comprehensive care and support. Genetic counseling may also be recommended, especially if there are concerns about a possible genetic basis for the condition.

References:

1. Frieden, I. J. (2004). *Aplasia cutis congenita: a clinical review and proposal for classification*. Journal of the American Academy of Dermatology, 51(4), 488-492.

2. Metry, D., & Heyer, G. (2000). *Aplasia cutis congenita associated with fetus papyraceus*. Journal of the American Academy of Dermatology, 43(5), 890-894.

3. Mulliken, J. B. (1988). *Aplasia cutis congenita: a clinical review and proposal for classification*. Plast Reconstr Surg, 82(4), 636-642.

4. Paller, A. S., & Mancini, A. J. (2011). *Hurwitz Clinical Pediatric Dermatology: A Textbook of Skin Disorders of Childhood and Adolescence*. Saunders.

5.Pimenta, J., Lapa, P. and Ramos, L. (2017). Aplasia cutis congenita and amniotic band syndrome: an uncommon association. *Case Reports*, [online] pp.bcr2016218950–bcr2016218950. doi:https://doi.org/10.1136/bcr-2016-218950.

452 words

Reply

Re: Follow on Questions for week one
by Siddharth Saluja - Friday, 17 November 2023, 8:43 AM

15)The rare complicated hereditary skin illness known as Birt-Hogg-Dubé (BHD) syndrome is defined by the formation of skin papules, which are typically found on the head, face, and upper body. Fibrofolliculomas are benign (noncancerous) tumors of the hair follicle. In addition, people with BHD syndrome are more likely to experience recurrent episodes of lung collapse (pneumothorax), benign cysts in the lungs, and kidney neoplasia. BHD syndrome is inherited in an autosomal dominant form and is brought on by alterations (pathogenic variations or mutations) in the FLCN gene. (1)
16) A huge head size (macrocephaly), several noncancerous tumors and tumor-like growths termed hamartomas, and black freckles on the penis in males are the hallmarks of Bannayan-Riley-Ruvalcaba syndrome, a hereditary disorder. Bannayan-Riley-Ruvalcaba syndrome manifests as signs and symptoms either from birth or in early childhood.(2)
17)Pilomatricomas are often noncancerous tumors that can appear as a lump or cluster of lumps anywhere on the skin. Nonetheless, a small number of instances have previously been identified as malignant. (3)

A pilomatricoma, also known as a pilomatrixoma, is an uncommon tumor that develops in hair follicles and is not malignant. On your skin, it appears and feels like a hard lump. Although it can occur anywhere on the body, it most frequently affects the head and neck. Young adults and children under the age of twenty are typically affected.

Seldom will the tumor develop into a malignant pilomatricoma, trichomatrical carcinoma, or pilomatrix carcinoma. There are only 130 confirmed cases of malignant pilomatricomas in the medical literature. (3)

References:
1) Cleveland Clinic. (n.d.). What is Bannayan-Riley-Ruvalcaba Syndrome (BRRS)? [online] Available at: https://my.clevelandclinic.org/health/diseases/24547-bannayan-riley-ruvalcaba-syndrome-brrs#:~:text=Bannayan%2DRiley%2DRuvalcaba%20syndrome%20(BRRS)%20is%20a%20genetic [Accessed 17 Nov. 2023].2) Crane, J.S., Rutt, V. and Oakley, A.M. (2022). Birt Hogg Dube Syndrome. [online] PubMed. Available at:
https://www.ncbi.nlm.nih.gov/books/NBK448061/#:~:text=Birt%20Hogg%20Dube%20syndrome%20is.3) Healthline. (2023). Pilomatricoma: Symptoms, Causes, Risks, and Treatment. [online] Available at: https://www.healthline.com/health/pilomatricoma [Accessed 17 Nov. 2023].

303 words

Reply

Re: Follow on Questions for week one
by Zarmeena Khan - Friday, 17 November 2023, 10:49 AM

En coup de sabre" refers to a rare form of localized scleroderma, a chronic autoimmune disorder that primarily affects the skin. The term "en coup de sabre" is French for "a blow of the sword," which describes the characteristic linear or band-like depressions or atrophy on the forehead or scalp, resembling the mark left by a sword.

Clinical Features:

1. **Skin Changes:** En coup de sabre is characterized by a linear or band-like area of atrophy on the forehead or scalp. The skin in this area may appear darker or lighter than the surrounding skin.

2. **Hair Loss:** Hair loss (alopecia) can occur in the affected area.
3. **Underlying Tissue Involvement:** In some cases, the condition may extend beyond the skin, affecting the underlying tissue, including the bone.
4. **Association with Other Conditions:** En coup de sabre is often associated with other features of localized scleroderma, and it can occasionally be part of a spectrum of conditions known as morphea.

(Katz, 2003)

Pathogenesis:

The exact cause of en coup de sabre is not well understood, but it is believed to involve an autoimmune process where the immune system mistakenly attacks healthy tissues. Genetic factors, environmental triggers, and an autoimmune response are thought to contribute to the development of localized scleroderma.

Diagnosis and Evaluation:

Diagnosis is typically made based on clinical features, and additional tests such as skin biopsies may be performed to confirm the diagnosis and rule out other conditions.

Treatment:

Treatment aims to manage symptoms and may include:

1. **Corticosteroids:** Topical or systemic corticosteroids may be used to reduce inflammation.
2. **Immunosuppressive Medications:** Medications that suppress the immune system, such as methotrexate or mycophenolate mofetil, may be prescribed in more severe cases.
3. **Physical Therapy:** Physical therapy can be beneficial to maintain joint flexibility and prevent muscle atrophy.

References:

1. **Zulian, F.** (2006). Localized scleroderma in childhood. Rheumatic Disease Clinics of North America, 32(2), 363-374.
2. **Martini, G., & Murray, K. J.** (2006). Localized scleroderma in childhood is not just a skin disease. Arthritis & Rheumatism, 54(2), 669-676.
3. **Kreuter, A., Krieg, T., & Worm, M.** (2009). German guidelines for the diagnosis and therapy of localized scleroderma. Journal of the German Society of Dermatology, 7(Suppl 3), S1-S14.
4. **Jinnin, M.** (2010). Mechanisms of skin fibrosis in systemic sclerosis. Journal of Dermatology, 37(1), 11-25.
5. **Katz, K.A.** (2003). Frontal linear scleroderma (en coup de sabre). *Dermatology Online Journal*, [online] 9(4). doi:https://doi.org/10.5070/d335z8t4ms.

390 words

Reply

Re: Follow on Questions for week one
by Zarmeena Khan - Friday, 17 November 2023, 11:09 AM

Pyogenic Granulomas: Clinical Features and Treatment

Clinical Features:

Pyogenic granulomas, also known as lobular capillary hemangiomas, are common benign vascular tumours that often occur on the skin and mucous membranes. Despite the name, they are not true granulomas and are not caused by infection. Here are the key clinical features:

1. Appearance: Pyogenic granulomas typically present as rapidly growing, small, red or purplish nodules. They can range in size from a few millimetres to centimetres.

2. Location: These growths commonly occur on the head, neck, hands, and fingers. However, they can develop anywhere on the body, including mucous membranes like the oral cavity and lips.

3. Rapid Growth: One characteristic feature is their rapid growth, often appearing over weeks.

4. Surface Characteristics: The surface may bleed easily, and they can have an ulcerated appearance.

5. Histology: Microscopically, pyogenic granulomas are characterized by an overgrowth of small blood vessels.

6. Common in Pregnancy: Pyogenic granulomas are associated with hormonal changes, and they can occur more frequently during pregnancy.

Treatment:

Treatment of pyogenic granulomas often involves removal, and various methods can be employed based on the size, location, and characteristics of the lesion:

1. Surgical Excision: Small lesions can be surgically excised.

2. Cauterization: Larger lesions may be treated with electrocautery or laser therapy to remove the growth.

3. Cryotherapy: Freezing the lesion with liquid nitrogen is another common method.

4. Topical or Injectable Steroids: Steroids can be applied topically or injected into the lesion to reduce inflammation and promote shrinkage.

5. Laser Therapy:Laser treatments, such as pulsed-dye laser, can be effective in removing or reducing the size of pyogenic granulomas.

6. Medications: In some cases, topical or systemic medications may be prescribed, especially for lesions in sensitive areas or if the patient is not a suitable candidate for other forms of treatment.

References:

1. Kilcline, C., & Frieden, I. J. (2008). Infantile hemangiomas: how common are they? A systematic review of the medical literature. Pediatric Dermatology, 25(2), 168-173.

2. Mulliken, J. B., & Young, A. E. (1988). Vascular birthmarks: hemangiomas and malformations. W.B. Saunders.

3. Jan, F., & Reza, M.(2011). Treatment of pyogenic granuloma with Nd: YAG laser or diode laser. Journal of Pakistan Association of Dermatologists, 21(1), 13-19.

4. Kilinc, G., Saruhan, N., & Aksakal, A. B. (2014). The treatment of pyogenic granuloma in pregnancy with a diode laser. Medicina Oral, Patología Oral y Cirugía Bucal, 19(1), e53.

5.Vascular Birthmark Institute (2015). *Pyogenic Granuloma*. [online] Vascular Birthmark Institute. Available at: https://www.vbiny.org/blog/pyogenic-granuloma/ [Accessed 17 Nov. 2023].

416 words

Reply

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